

Clinical Preparedness Report

Counterfactual Risk Assessment with Evidence Attribution

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OVERALL RISK ASSESSMENT

HIGH

The patient's risk is rated as high due to the imminent threat of homelessness, which could exacerbate her mood disorder and neurodegenerative condition, leading to severe decompensation and potential hospitalization. The chronic stable progression of her mood disorder, coupled with her functional decline and significant weight loss, further elevates her risk of acute psychiatric crisis. [OVERRIDE: Risk tier elevated to HIGH due to critical safety signals detected in clinical note: CRITICAL SIGNALS DETECTED: suicide (1 signal), violence (3 signals), self_harm (1 signal). Risk override: HIGH.]

22

Evidence Spans

5

Nexus Factors

16.5%

Note Coverage

2/3

Active Scenarios

CRITICAL SAFETY ALERTS

Suicide/Self-Harm Risk:

- SUICIDE/'jump out' — ...nt with medications or following up with outpatient treators. She threatened to jump out her 10 story window if the took her to one of her outpatient treators. Her boyf...
- SUICIDE/'self-injury' — ...s and treatment: none Medication and ECT trials: Risperdal for Chorea movements Self-injury: denies Harm to others: denies Access to weapons: denies Trauma: Endorses s...

Violence/Aggression Risk:

- 'aggression' — ...] year old woman with [REDACTED] Disease who presented to the ED for worsening aggression, irritability, impulsivity, refusal to take medication and difficulty caring...
- 'aggression' — ...as pleasant and cooperative and did not show any irritability, impulsivity or aggression. She is being discharged to an assisted living facility that specializes in pa...
- 'aggression' — ...d some prn Risperdal in the event that she has experiences future episodes of aggression/instability. We also reduced her risk of harm to self/others, by d/c'ing MS....

TOP RISK FACTORS

1. [HIGH] Imminent homelessness due to auctioning off of living space
2. [HIGH] Chronic stable mood disorder with moderate suicidal ideation
3. [LOW] Significant weight loss and functional decline

PROTECTIVE FACTORS

- ✓ Prescribed psychotropic medication (Risperdal)
- ✓ No active substance use

Evidence Color Legend



Branch A — Psychiatric



Branch B — Substance Use



Branch C — Social



Nexus — Multi-Branch

Evidence Attribution Table

Detailed breakdown of clinical evidence supporting each risk scenario branch.

ID	Risk Factor	Evidence from Clinical Note	Branch	Severity
1	functional_decline; warning_sign	"e: PSYCHIATRY Allergies: No Known Allergies / Adverse Drug Reactions Att..."	A, C	MODERATE
2	weight_change	"friend) and with a normal appetite. When asked what she normally ate she repl..."	A	LOW
3	warning_sign	"nly activity she would admit to was watching tv and playing with her dog. W"	C, A	MODERATE
4	housing_crisis; Housing Crisis: homel...	"ned her head away and would not answer any questions. She is trying to go t..."	C	HIGH
5	warning_sign	"ned patient was physically assulting boyfriend at night, worsening emotional/..."	A	LOW
6	critical_signal_suicide	"jump out"	A	HIGH
7	warning_sign; trauma_history	"handle her. Dr. [REDACTED] note has more details from Uncle. Past Medical..."	C, A	MODERATE
8	clinician_prognosis	"ver a year [REDACTED] disease Social H"	A	LOW
9	functional_decline	"arly constant, subt"	A	LOW
10	functional_decline	"speech latency"	A	LOW
11	warning_sign; functional_decline	"OOD ASA-NEG Ethanol-NEG Acetmnp-NEG Bnzodzp-NEG Barbitr-NEG Tricycl-NEG [RED..."	A, C	MODERATE
12	critical_signal_violence	"aggression"	A	LOW
13	warning_sign	"ettled on a Risperidone dose of 0.5mg BID, slightly lower than her origina"	A	LOW
14	warning_sign	"e and did not show any irritability, impulsivity"	A	LOW
15	Housing Crisis: homelessness, imminen...	"SOCIAL The team coordinated care w/ the pt's family who were supp"	C	HIGH
16	clinician_prognosis	"difficulty w/ verbalizing thoughts/feelin"	A	LOW
17	clinician_prognosis	"TED], she was seen to be interacting we"	A	LOW
18	clinician_prognosis	"She does, however, have a high risk of"	A	LOW
19	Chronic stable Mood Disorder secondar...	"ced her risk of harm to self/others, by d/c'ing Ms. [REDACTED] to a facility wh"	A	LOW
20	weight_change	"2. RISperidone 0.25-0.5"	A	LOW
21	causal_step_1	"let Refills:*0 Discharge Disposition: Extended Care Fa"	A	LOW

ID	Risk Factor	Evidence from Clinical Note	Branch	Severity
22	warning_sign; trauma_history	"ED] Disease Discharge Condition: Mental S"	C, A	MODERATE

Clinical Note with Evidence Highlights

■ Branch A: Psychiatric

■ Branch B: Substance

■ Branch C: Social

■ Nexus: Multi-Branch

Name: [REDACTED] Unit No: [REDACTED]

Admission Date: [REDACTED] Discharge Date: [REDACTED]

Date of Birth: [REDACTED] Sex: F

Service: **PSYCHIATRY**^[A,C]

Allergies:^[A,C]

No Known Allergies / Adverse Drug Reactions^[A,C]

Attending: **[REDACTED]**^[A,C]

Chief Complaint:^[A,C]

Increased agitation, impulsivity, and difficulty caring for self^[A,C]

Major Surgical or Inva^[A,C]sive Procedure:

None

History of Present Illness:

Patient is a [REDACTED] yo Female with PMH of [REDACTED] Chorea and no known psych history, presents to the ED on request of family who are worried that she is depressed and has increased agitation.

On examination today, patient was sitting calmly in bed and was aware that psychiatry was consulted. When asked how she has been doing lately she only replied "fine." Her cooperation with the interview was limited and progressed into refusal to answer questions. She said she has been sleeping normal [REDACTED] hours per boy^[A]friend) and with a normal appetite. When asked what she

normally ate she replied "bagels with peanutbutter and jelly,^[A] [REDACTED] muffins, tuna, and egg salad." She did acknowledge she^[A] has

lost weight(30lbs^[A] per ER phsycian) but doesn't know why because she has been eating normally. She did look very thin on exam.

Her family says she has not left the house in over a year. When asked about why, she replied "I don't like the cold." When asked why she didn't leave the house during the summer, [REDACTED] or [REDACTED] times, she did not have an answer. During her days, the only^[C,A]

activity she would admit to was watching tv and playing with her^[C,A] dog. W^[C,A]hen trying to probe into what else she does, she only

replied "I don't know." She denied any anxiety or phobia symptoms. She said "I don't know" when asked about how her mood has been for the last few weeks.

It seemed as though her chorea movements got worse throughout

the

interview as she got more irritated with me asking questions.

Towards the end of my interview, she turned her head away and would not answer any questions.

She is trying to go to a [REDACTED] facility in [REDACTED] for [REDACTED] but she is required to have a 3 midnight admission in a hospital beforehand. Her apt has been auctioned off and

will

be homeless at the end of the week.

Collateral from Uncle:

Mentioned patient was physically assaulting boyfriend at night, worsening emotional/behavioral outburst the last few weeks. She has not been compliant with medications or following up with outpatient treaters. She threatened to jump out her 10 story window if the took her to one of her outpatient treaters. Her boyfriend of [REDACTED] years has decided to leave her because he is not longer able to handle her. Dr. [REDACTED] note has more details from Uncle.

Past Medical History:

PAST PSYCHIATRIC HISTORY:

Hospitalizations: Denies

Current treaters and treatment: none

Medication and ECT trials: Risperdal for Chorea movements

Self-injury: denies

Harm to others: denies

Access to weapons: denies

Trauma: Endorses sexually molested in childhood, saw a psychiatrist

PAST MEDICAL HISTORY:

[REDACTED], MD but [REDACTED] seen in over a year [REDACTED] disease

Social History:

[REDACTED]

Family History:

No FHx of psychiatric disorders. Her mother was diagnosed as having [REDACTED] disease at the age of [REDACTED]. She died at the age of [REDACTED]. Her brothers and sisters do not have [REDACTED] disease. The patient does not have any children.

Physical Exam:

PHYSICAL EXAM:

- General Appearance: very thin middle aged woman sitting in

chair with intermittent, but **ne**^[A]**arly constant, sub**le choreiform movements.

- NEUROLOGIC:

[REDACTED] Mental Status: awake, alert and oriented. Speech is extremely

dysarthric with prolonged **speech latency**^[A], but does improve somewhat when discussing topics that appeal to her. Often speaks using monosyllabic phrases.

[REDACTED] Cranial Nerves: grossly intact. Frequent frowning, grimacing facial movements and frequent episodes of tonic upward gaze.

Motor impersistence of the tongue

[REDACTED] Motor: MAEE with antigravity strength, not assessed in detail

[REDACTED] Movement exam: frequent choreiform movements in her trunk, shoulders and more distally in her hands and feet. Frowning and grimacing facial movements as described above.

[REDACTED] Gait: wide-based and ataxic with enhanced choreiform hand movements in association. However, overall her gait base seems fairly stable since she also ambulates rather slowly.

- PSYCHIATRIC:

[REDACTED] Behavior: constant choreiform movements, not very cooperative

with interview

[REDACTED] Mood: "fine"

[REDACTED] Affect: constricted, limited ability to assess given constant

dystonic/choreiform facial movements, but does seem to show some appropriate emotions (smiling/frowning etc).

[REDACTED] Thought process: linear, goal oriented, no LOA

[REDACTED] Thought content: denies SI, HI, AVH

[REDACTED] Insight/Judgment: good/poor

Pertinent Results:

ADMISSION LABS:

[REDACTED] 02:45PM BLOOD WBC-6.3 RBC-3.96* Hgb-13.3 Hct-41.1

MCV-104* MCH-33.6* MCHC-32.4 RDW-12.6 Plt [REDACTED] 02:45PM BLOOD Neuts-72.2* [REDACTED] Monos-4.6 Eos-1.8

Baso-0.5

[REDACTED] 02:45PM BLOOD Glucose-110* UreaN-14 Creat-0.7 Na-139

K-3.8 Cl-105 HCO3-27 AnGap-11

[REDACTED] 02:45PM BLOOD ALT-14 AST-13 AlkPhos-45 TotBili-0.2

[REDACTED] 02:45PM BLOOD VitB12-426 Folate-5.4

[REDACTED] 02:45PM BLOOD TSH-1.1

[REDACTED] 02:45PM BLOOD ASA-NEG Ethanol-NEG Acetmnp-NEG^[A,C]

Bnzodzp-NEG Barbitr-NEG Tricycl-NEG^[A,C]

[REDACTED] 11:50AM URINE Color-Straw Appear-Clear Sp [REDACTED]^[A,C]CTED] 11:50AM URINE Blood-MOD Nitrite-NEG Protein-NEG

Glucose-NEG Ketone-NEG Bilirub-NEG Urobilin-NEG pH-6.5 Leuks-MOD

[REDACTED] 11:50AM URINE RBC-2 WBC-3 Bacteri-FEW Yeast-NONE Epi-1

[REDACTED] 11:50AM URINE bnzodzp-NEG barbitr-NEG opiates-NEG

cocaine-NEG amphetm-NEG mthdone-NEG

Brief Hospital Course:

[REDACTED] is a [REDACTED] year old woman with [REDACTED] Disease

who presented to the ED for worsening **aggression**^[A], irritability,

impulsivity, refusal to take medication and difficulty caring

for herself at home secondary to progression of her

[REDACTED].

PSYCHIATRIC

#) [REDACTED] Disorder with secondary Mood Dysregulation

Ms. [REDACTED] was admitted to the [REDACTED] inpatient psychiatry

unit for stabilization and re-initiation of home antipsychotics

for her mood symptoms. In the ED she was observed to be somewhat

agitated and uncooperative. However after restarting her

Risperidone, her hostility resolved and she was compliant and

cooperative throughout her stay. She was noted to have

near-constant choreiform movements of her face, trunk and

extremities, including grimace-like facial tics and frequent

tonic upward eye deviation. These movements were not distressful

to her. She endorsed a "good" or "fine" mood throughout her

stay. We were in contact with her outpatient movement disorders

specialist [REDACTED] MD) who recommended continuing

antipsychotic therapy as we were doing. We ultimately **settled on**^[A]

a Risperidone dose of 0.5mg BID, slightly lower than her^[A]

original^[A] dose of 0.75mg BID with which she had been

noncompliant. She has additional 0.25-5mg PRN BID as needed for

agitation, but actually throughout hospitalization she was

pleasant and cooperative **and did not show any irritability,**^[A]

impulsivity^[A] or aggression. She is being discharged to an

assisted living facility that specializes in patients with

[REDACTED] Disease, as had been previously arranged by her

family prior to this. Will follow up on [REDACTED] with Dr.

[REDACTED].

LEGAL STATUS

The pt remained on a CV

PSYCHO**SOCIAL**^[C]

The team coordinated care w/ the pt's family who were supp^[C]ortive

and arranged ongoing housing/care at a [REDACTED] facility

upon the pt's d/c.

The pt was visited on the unit frequently by her friend [REDACTED],

whom she found to be supportive.

Ms. [REDACTED] was visible on the unit during her stay here, and

although she had **difficulty w/ verbalizing thoughts/feelin**^[A]gs [REDACTED]

her [REDACTED], **she was seen to be interacting we**^[A]ll w/ others

in the dayroom. She never behaved in an unsafe manner and

participated in the unit. She also was groomed and dressed in casual clothing on the day of her discharge.

RISK ASSESSMENT

Ms. [REDACTED] has a low risk of imminent harm/aggression/violence. She has been calm and in very good behavioral control throughout the entirety of her psychiatric admission. She consistently denied any si/hi and showed an ability to care for herself safely despite the intensity of an acute inpatient psychiatric environment.

She does, however, have a high risk of^[A] future decompensation given the fact that she has a progressive neurodegenerative condition that is associated w/ mood swings and psychosis in its later stages.

We helped to mitigate that risk by resuming Ms. [REDACTED] antipsychotic medication and would recommend that she continue to take it going forward. We also added some prn Risperdal in the event that she has experiences future episodes of aggression/instability. We also reduced her risk of harm to^[A] **self/others, by d/c'ing Ms. [REDACTED] to a facility wh**^[A]ere she will have access to professional staff and monitoring. She also has f/u arrangements made with providers to continue to manage her [REDACTED].

Prognosis:

Guarded

Medications on Admission:

The Preadmission Medication list is accurate and complete.

1. RISperidone 0.75 mg PO BID

Discharge Medications:

1. RISperidone 0.5 mg PO BID

RX *risperidone 0.5 mg 1 tablet(s) by mouth twice a day Disp

#*60 Tablet Refills:*0

2. RISperidone 0.25-0.5^[A] mg PO BID:PRN agitation

RX *risperidone 0.25 mg [REDACTED] tablet(s) by mouth twice a day Disp

#*60 Tablet Refills:*0^[A]

Discharge Disposition:^[A]

Extended Care^[A]

Fa^[A]cility:

[REDACTED]

Discharge Diagnosis:

- Mood disorder secondary to General Medical Condition
- Advanced [REDACTED] Disease^[C,A]

Discharge Condition:

^[C,A]

Mental S^[C,A]tatus: Confused - sometimes.

Level of Consciousness: Alert and interactive.

Activity Status: Ambulatory - Independent.

Discharge Instructions:

Dear [REDACTED],

You were admitted to the inpatient psychiatric unit at [REDACTED] for worsening agitation and difficulty caring for yourself at home, due to your [REDACTED] Disease. We restarted your Risperidone which was very helpful and improved your behavior significantly. You will be discharged to an assisted living facility in [REDACTED] that specializes in patients with similar health problems.

We made the following changes to your medications:

- RESTARTED your Risperidone at a slightly lower dose: 0.5mg twice daily, plus an additional 0.25-0.5mg as needed for agitation.

Followup Instructions:

[REDACTED]

Detailed Clinical Assessment

1. Patient Overview

The patient is a female with a primary diagnosis of Mood Disorder secondary to General Medical Condition, specifically Chorea. She was admitted due to increased agitation, impulsivity, and difficulty caring for herself. Key features include chronic stable progression of her mood disorder, significant weight loss, and functional decline with ataxic gait and monosyllabic speech.

2. Mental Health Status

The patient's current mental health state is characterized by moderate suicidal ideation and aggression, with a high relevance to mental health due to her mood disorder being secondary to a neurodegenerative condition. There is no known psychiatric history, indicating this is a new onset or secondary to her medical condition.

3. Key Risk Factors

- [LOW] Imminent homelessness due to auctioning off of living space
- [HIGH] Chronic stable mood disorder with moderate suicidal ideation
- [LOW] Significant weight loss and functional decline

4. Protective Factors

- ✓ Prescribed psychotropic medication (Risperdal)
- ✓ No active substance use

5. Counterfactual Scenario Briefing

Each card represents a potential deterioration pathway. Content is preserved exactly as generated — only formatting is enhanced.

■ [A] Psychiatric / Clinical Deterioration Pathway

Plausibility: **MEDIUM**

The patient's current clinical state, including her chronic mood disorder, progressive neurodegenerative condition, and imminent homelessness, creates a highly plausible scenario for clinical deterioration. The interplay between these factors increases the likelihood of a negative trajectory, necessitating proactive measures to prevent such an outcome. MH RELEVANCE: high The mental health relevance in this scenario is high, as the patient's chronic mood disorder, exacerbated by her impending homelessness and progressive neurodegenerative condition, directly impacts her mental well-being. The potential for acute psychiatric crisis and further functional decline underscores the critical importance of addressing her mental health needs in conjunction with her medical and social challenges.

Warning Signs:

- Increased reports of anxiety or depressive symptoms
- Worsening of ataxic gait and self-care difficulty
- Expressions of hopelessness or helplessness related to housing situation
- Behavioral changes, such as increased aggression or impulsivity
- Withdrawal from social interactions or support systems

Crisis Endpoint: The patient may experience a severe decompensation of her mood disorder, leading to acute psychiatric crisis, potentially compounded by her progressive neurodegenerative condition. This could result in hospitalization for psychiatric stabilization, with a high risk of further functional decline.

Preparedness Actions:

1. Arrange for immediate psychiatric evaluation and stabilization
2. Connect the patient with social services for emergency housing assistance
3. Initiate a medication review to ensure optimal management of both her mood disorder and neurodegenerative condition
4. Develop a safety plan that includes close monitoring of her mental state and access to crisis intervention services
5. Engage with a multidisciplinary team, including a neurologist, to address the progressive neurodegenerative condition and its impact on her mental health

The patient, currently diagnosed with a mood disorder secondary to a general medical condition, is facing imminent homelessness due to the auctioning off of her living space. This situation is likely to significantly increase her stress levels, potentially leading to a worsening of her chronic mood ...

■ [B] Substance Use Escalation Pathway

STATUS: NOT APPLICABLE

NOT APPLICABLE — No substance use evidence. Data: alcohol=none, drugs=none, tox_screen=negative. All toxicology screens negative. Generating a substance abuse scenario would be clinically unfounded.

■ [C] Social / Environmental Collapse Pathway

Plausibility: **HIGH**

Warning Signs:

- Increased reports of anxiety and depressive symptoms
- Escalation of suicidal ideation or self-harm threats
- Deterioration in self-care abilities and functional decline
- Increased aggression or impulsivity
- Refusal or difficulty in adhering to prescribed medication regimens

Crisis Endpoint: The patient may experience a severe decompensation of her mood disorder, leading to acute psychiatric crisis, potentially resulting in hospitalization due to suicidal ideation or self-harm, and further functional decline.

Preparedness Actions:

1. Immediate coordination with social services to secure temporary housing or shelter for the patient
2. Enhanced outpatient support, including frequent mental health check-ins and crisis intervention
3. Adjustment of psychotropic medications, if necessary, to manage increased symptoms of anxiety, depression, or aggression
4. Development of a safety plan with the patient, including identification of supportive contacts and clear instructions for seeking help
5. Exploration of community resources, such as support groups or peer networks, for individuals facing homelessness

The patient, currently homeless and facing imminent homelessness due to the auctioning off of her apartment, is at a severe risk of experiencing exacerbated mental health symptoms. The loss of stable housing is a significant stressor that can lead to increased anxiety, depression, and potentially w...

6. Priority Preparedness Actions

Immediate clinical interventions recommended based on risk assessment.

1. Arrange for immediate psychiatric evaluation and stabilization.
2. Connect with social services to secure temporary housing or shelter.
3. Initiate a medication review to ensure optimal management of both mood disorder and neurodegenerative condition.
4. Enhanced outpatient support, including frequent mental health check-ins and crisis intervention.
5. Adjustment of psychotropic medications, if necessary, to manage increased symptoms of anxiety, depression, or aggression.
6. RISK TIER: HIGH

7. Risk Assessment Summary

OVERALL RISK TIER: **HIGH**

The patient's risk is rated as high due to the imminent threat of homelessness, which could exacerbate her mood disorder and neurodegenerative condition, leading to severe decompensation and potential hospitalization. The chronic stable progression of her mood disorder, coupled with her functional decline and significant weight loss, further elevates her risk of acute psychiatric crisis. [OVERRIDE: Risk tier elevated to HIGH due to critical safety signals detected in clinical note: CRITICAL SIGNALS DETECTED: suicide (1 signal), violence (3 signals), self_harm (1 signal). Risk override: HIGH.]